ENSIGN SERVICES

POSTette: Understanding Justification of Skilled Services

Reference: Medicare Benefit Manual Chapters 8 & 15, ASHA, AOTA, APTA

Rehabilitation documentation must support medical necessity of therapy services provided and answer the question: Are the services appropriate for the patient's condition and do the services require the skills and knowledge of a qualified clinician?

Documentation needs to indicate the clinician's unique professional contribution to the therapy services:

- Why did the patient require professional treatment, education or training?
- What specialized treatment, education or training did the clinician actually provide?
- How did the patient benefit from the specialized knowledge applied by the clinician?

Capturing Justification for Services on the Evaluation

Even though a patient's medical condition is a valid factor in deciding if skilled therapy services are needed, a patient's diagnosis or prognosis is never the sole factor in deciding that a service is or is not skilled. **The key issue is whether the skills of a therapist are needed to treat the illness or injury, or whether the services can be carried out by non-skilled personnel after sufficient training.**

The documentation must describe the patient's condition before, during, and after the therapy episode to support that the patient significantly benefitted from ongoing therapy services and that the progress was sustainable and of practical value when measured against the patient's condition at the start of treatment.

Documentation of comparable objective/functional measures (standardized test results compared to where patient needs to be at DC / PLOF) plays a key role in demonstrating medical necessity.

- Dx that line up and make sense
- Detailed PLOF
- Reason for referral and history that makes sense
- Thorough assessments and impairments that align with the condition

NOTE: Objective/functional measures must minimally be established for the patient's prior level of function (PLOF), status at the initial evaluation, and status at each progress reporting interval.

Capturing Justification on the POC

The Plan of Care must identify goals that would benefit the patient (or train caregivers) functionally. When developing goals consider if the patient have a practical reason to meet the goal and does the goal align with the severity of the impairments and the medical condition?

Goals need to be patient driven, directly tied to functional limitations, related to a functional activity that can be measured, and have a specified time frame. When establishing patient goals, it is helpful to tie back to the deficits identified on the evaluation and results of standardized testing. (Do not develop goals for any area indicated as DNT or Assessment not clinically indicated)

Reasonable and Necessary

What is the reason for intervention?
Establish goals to address the issue. What steps will be used to improve language/ mobility/ ADLs?

Reasonable – the objective relates in a realistic manner to the potential the patient possesses, and are achievable in a period of time that corresponds to the severity of impairment and the potential for recovery.

Necessary- there is a reason the patient needs to accomplish the goal.

- Elicit positive medical/functional improvement (function must be tied to the expectation to perform the activity – people living in SNF's don't need to manage money, people living with 24-hr caregivers have available supervision, etc.)
- Prevent adverse medical impact/increased dependence related to the documented impairment

Frequency and Duration

The frequency and duration of services must be justifiable according to the documented severity of the patient's condition, responsiveness to treatment, and demonstrated change in function

Documentation needs to establish the variables that influence the patient's condition, especially those factors that influence the clinician's decision to provide more services than are typical for the individual's condition. Clinicians shall determine typical services using published professional literature and professional guidelines. Services that exceed those typically provided for any given condition, need to be carefully documented to justify their necessity.

Justification statement are included within the documents to support reasoning for providing services that are more extensive than is typical for the condition treated.

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Capturing Justification in the Progress Reports

Documentation needs to demonstrate through objective measurements that the patient is making progress toward goals.

Note: Regression and plateaus can occur during treatment. It is essential that the reasons for lack of progress and the justification for continued treatment are documented if treatment continues after regression or plateaus.

Be careful how comparative measures are reported. For example:

- Acceptable comparative measure: At the time of the initial evaluation the patient had an objective manual muscle test (MMT) of 3/5 for right knee extension. Documentation in the progress report supported that the patient had achieved 4/5 for right knee extension which demonstrated significant benefit for the patient.
- Unacceptable comparative measure: At the time of the progress report the documentation supported that the patient had an objective MMT of 4/5 for right biceps flexion. At the time of the next progress report documentation supported that the patient was able to complete 3 sets of 10 repetitions for right biceps flexion during each therapy visit. Since the MMT information is not directly comparable to the therapeutic exercise sets and repetitions this comparison does not clearly demonstrate ongoing significant benefit for the patient and may result in denial.

Material Impact

One of the biggest challenges on review is not being able to demonstrate the material impact when progress isn't being made. Skilled intervention can be justified with documentation of:

- **Assessment and analysis** of patients performance and adjustments to the plan (using standardized tests, measuring vitals with activity vs. at rest, analysis of task performance of when to make it harder/ less challenging and why, etc.)
- **Prevention of decline or deterioration** through various skilled intervention approaches individualized for each patient
- **Training others to facilitate improvement or prevent decline** (care planning for nursing carry over, development of restorative programming, preparing family members for a safe transition of care, etc.)
- **Decreased medical risk** (what is the risk?) through specialized skilled intervention approaches (assessment of vitals to determine safe levels of physical activity, exercise prescription based on patients levels of performance, functional balance assessments and interventions designed to reduce fall risks, swallow abilities and diet modifications to decrease aspiration or malnutrition, self-care strategies and endurance training, etc.)

Patients who need therapy generally respond to therapy, so changes in objective measures of improvement also help establish the need for services. The use of scientific evidence, obtained from professional literature, and sequential measurements of the patient's condition during treatment is encouraged to support the potential for continued improvement that may justify the patient's need for therapy.

Justification Statements - Why does skilled intervention need to continue?

- What skills were demonstrated/ achieved during the progress note reporting period (if not clear in the goals summary additional information needs to be added to support continuation of services.)
- What deficits remain that can benefit from skilled intervention
- What services will be provided to support continuation of skilled intervention

Examples of supportive justification statements:

"The patient has transitioned from honey thick liquids to nectar thick liquids and has increased the number of CCs consumed at meals. The patient is unable to advance to thin liquids due to decreased laryngeal strength and delayed swallow. Continued speech-language pathology services are medically necessary to improve laryngeal strength and function; train compensatory swallow strategies and assess ability to tolerate advanced textures without signs/symptoms of aspiration."

"Patient has made improvement with LE strength, sensation, gait quality and balance. Patient has not had any falls since initiation of intervention. Goals 1 and 3 have been met. Patient continues to demonstrate inability to progress to a single point cane and demonstrates loss of balance during high level gait activities. Continued physical therapy is medically necessary to improve gait patterns and balance to progress to independent mobility to discharge to prior level of function. New goal of _____ will be added to plan of care."

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Capturing Justification in the Treatment Encounter Notes

Successful rehabilitation therapy requires the attainment of significant progress for a reasonable period of time beyond the immediate intervention. Services should not be repetitive, palliative, or simply reinforcing previously learned skills or maintaining function. In most instances, the goal of therapy is to maximize patient/caregiver abilities within the patient's home environment.

- Description of skilled activities that are patient specific
- Details of patient response
- Documentation of caregiver involvement

Understanding the needs of the patient is supported by:

- Knowledge of the individual patient's condition
- Any complexities that impact that condition
- The patient's diagnoses
- Complicating factors, age, severity, time since onset/acuity, self-efficacy/motivation, cognitive ability, prognosis, and/or medical, psychological and social stability.

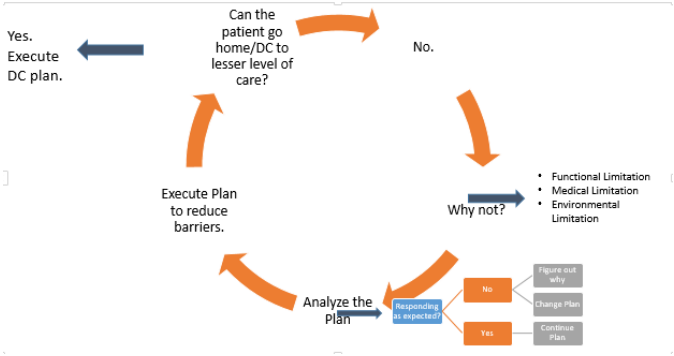
Capturing Justification in the UPOC

A UPOC justifies the need for continued skilled intervention using the data from the original evaluation, original POC, and weekly progress reports, in order to establish why continued services are reasonable and necessary. The UPOC is the clinician's opportunity to describe why skilled intervention continues to be indicated and how the patient will benefit from additional therapy

Why are services continuing?

- Because the patient has not yet met the goals and is responding well to treatment, such that meeting the goals is expected.
- Because the patient has not yet met the goals and did not respond well to treatment, but based on changes (to the goals or the treatment plan), it is reasonable to expect progress now.
- Because new areas have been identified for remediation that are reasonable and necessary.

Whichever statement best reflects the reason to continue the plan of care must be supported by data that is specific to the patient and the treatment plan.



Capturing Justification in the Discharge Summary

A discharge summary is required for every episode of treatment. Emphasis needs to be on describing the medical necessity and justification of services provided especially any services provided beyond those usually expected for any given condition.

- Provide a description of any complicating factors that impacted progress.
- Take the time to emphasize the skilled services and the treatment methods provided.
- Conclude with a brief statement of how skilled intervention has improved the patient's function and/or quality of life.

Strong justification statements enhance the clinical picture and are critical to supporting skilled intervention.

In the event of a record review, the information documented is the only evidence of what the deficits were, what interventions were provided, the patient's response to those interventions, and the functional status at discharge.

If therapists do not capture this information at the time services occurred, then they run the risk of having the services denied by a reviewer. The most common reasons for denials outside of "technical issues" are: services are not skilled, no statement of functional goals, amount/frequency/duration was not reasonable, or documentation does not support the minutes logged.

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